

Preeclampsia Prevention Audit Worksheet

Approved extractor only / one encounter per sheet / no chart identifiers

Proposed paper-workflow data collection companion. Confirm eligibility, denominators, repeat-visit handling, and coding with the investigator before use. Do not include names, dates of birth, MRNs, exact encounter dates, signatures, or identifying free text.

A. STUDY LINKAGE - managed within the approved extraction process

Approved study ID: _____ Encounter sequence: _____ Study week: _____

Extraction completeness: Complete / Incomplete / Unable to assess

B. ORIGINAL PROJECT DOMAINS - keep missing separate from No

Check ONE answer per row; blank = not completed.

	Yes	No	Missing
Risk screening completed and documented	☐	☐	☐
Counseling documented when indicated	☐	☐	☐
Aspirin decision documented	☐	☐	☐

Counseling not applicable with documented reason? Yes / No / Unknown

GA at screening: _____ weeks _____ days / Missing

GA at decision: _____ weeks _____ days / Missing

C. DECISION CODE - transcribe the clinician record; do not infer

☐ Initiated ☐ Continued ☐ Not indicated ☐ Declined

☐ Contraindicated ☐ Held ☐ Pending ☐ Planned start ☐ Missing

Exception codes supplement the four original project categories. Do not silently classify contraindicated, held, pending, or planned-start encounters as not indicated. Investigator must predefine research mapping.

Dose documented: _____ mg / Missing / N/A

GA at actual initiation: _____ weeks _____ days / Unknown / N/A

D. QUALITY CHECK - proposed supplemental fields

☐ Clinician signature present ☐ Paper form filed ☐ Incomplete record flagged

Clinical eligibility category: High risk / Multiple moderate / Consideration / None / Unknown

Contraindication review documented: Yes / No / Missing

Reason for justified exception documented: Yes / No / Missing / N/A

Report process measures separately. A prescription does not establish actual medication use. This worksheet does not measure preeclampsia outcomes or prove causality.

E. PRIVACY AND COUNTING RULES

Use an approved study ID. Any linkage key stays under the approved practice data process, separate from the research extract. Suppress identifying details and small-cell disclosures as required. Do not upload completed worksheets to the public website.

Suggested encounter-level reporting: completed assessments / eligible encounters; documented counseling / encounters requiring counseling; documented decisions / reviewed encounters. Patient-level reporting requires a prespecified repeat-visit rule. All definitions require investigator agreement.